[redacted]

Visit ID	: RCI24395	Registration	: 24/Jan/2026 12:49PM
UHID/MR No	: RCI.0000021604	Collected	: 24/Jan/2026 12:50PM
[redacted]		Received	: 24/Jan/2026 12:50PM
Age/Gender	: 58 Y 0 M 0 D /M	Reported	: 24/Jan/2026 01:53PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	:	Client Code	: 320
Client Add	:	Barcode No	: 10082636

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
ERYTHROCYTE SEDIMENTATION RATE				
Sample Type : WHOLE BLOOD EDTA				
ERYTHROCYTE SEDIMENTATION RATE	68	mm/hr	<12 mm/hr	Westergren

COMMENTS: ESR is an acute phase reactant that indicates the presence and intensity of an inflammatory process. It is never diagnostic of a specific disease. It is used to monitor the course or response to treatment of certain diseases. Extremely high levels are found in cases of malignancy, hematologic diseases, collagen disorders, and renal diseases. - Increased levels may indicate: Chronic renal failure (e.g., nephritis, nephrosis), malignant diseases (e.g., multiple myeloma, Hodgkin disease, advanced Carcinomas), bacterial infections (e.g., abdominal infections, acute pelvic inflammatory disease, syphilis, pneumonia), inflammatory diseases (e.g. temporal arteritis, polymyalgia rheumatic, rheumatoid arthritis, rheumatic fever, systemic lupus erythematosus [SLE]), necrotic diseases (e.g., acute myocardial infarction, necrotic tumor, gangrene of an extremity), diseases associated with increased proteins (e.g., hyperfibrinogenemia, macroglobulinemia), and severe anemias (e.g., iron deficiency or B12 deficiency).
Falsely decreased levels may indicate Sickle cell anemia, spherocytosis, hypofibrinogenemia, or polycythemia vera.

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Visit ID	: **RCI24354**	Registration	: 23/Jan/2026 03:07PM
UHID/MR No	: RCI.0000021563	Collected	: 23/Jan/2026 03:09PM
Patient Name	: [redacted]	Received	: 23/Jan/2026 03:13PM
Age/Gender	: 58 Y 0 M 0 D /M	Reported	: 23/Jan/2026 05:01PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	:	Client Code	: 320
Client Add	:	Barcode No	: 10082595

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
CBC				
Sample Type : WHOLE BLOOD EDTA				
HAEMOGLOBIN (HB)	13.6	g/dl	13.0-17.0	CYANMETHEMOGLOBIN
TOTAL LEUCOCYTE COUNT (TLC)	7.55	10^3/uL	4.0-10.0	Flow cytometry
NEUTROPHIL	80	%	40-80	
LYMPHOCYTE	**13**	%	20-40	
MONOCYTE	06	%	2-10	
EOSINOPHIL	01	%	1-6	
BASOPHIL	00	%	<2	
ABSOLUTE NEUTROPHIL COUNT	6.04	10^3/uL	2.0-7.0	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	0.98	10^3/uL	1.0-3.0	Automated Calculated
ABSOLUTE MONOCYTE COUNT	0.45	10^3/uL	0.2-1.0	Automated Calculated
ABSOLUTE EOSINOPHIL COUNT	0.07	10^3/uL	0.02-0.5	Automated Calculated
ABSOLUTE BASOPHIL COUNT	**0.00**	10^3/uL	0.02-0.1	Automated Calculated
RBC COUNT(RED BLOOD CELL COUNT)	**4.18**	million/µL	4.7-6.1	Optical Flowcytometry
PCV/HAEMATOCRIT	40.8	%	40-45	RBC pulse height detection
MCV	97.8	fL	80-100	Automated/Calculated
MCH	32.6	pg	26-34	Automated/Calculated
MCHC	33.3	g/dl	32-36	Automated/Calculated
RDW-CV	**11.3**	%	11.6-14.0	Automated/Calculated
RDW-SD	**46.2**	fL	39-46	Calculated
PCT	0.282	%	0.108-0.282	
MPV	11.0	fL	7.0-12.0	Calculated
PDW	15.6	fL	9-17	Calculated
PLATELET COUNT	255	10^3/uL	150-410	Optical Flowcytometry

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Visit ID	**: RCI24354**	Registration	: 23/Jan/2026 03:08PM
UHID/MR No	: RCI.0000021563	Collected	: 23/Jan/2026 03:09PM
Patient Name	: [redacted]	Received	: 23/Jan/2026 03:13PM
Age/Gender	: 58 Y 0 M 0 D /M	Reported	: 23/Jan/2026 05:01PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	:	Client Code	: 320
Client Add	:	Barcode No	: 10082595

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
EXTENDED KIDNEY FUNCTION TEST				
Sample Type : SERUM				
SERUM UREA	20.2	mg/dl	16.8-43.2	Urease GLDH
SERUM CREATININE	0.80	mg/dL	0.8-1.3	Jaffe
SERUM URIC ACID	4.70	mg/dl	3.6-8.2	URICASE
Blood Urea Nitrogen (BUN)	9.62	mg/dl	5-25	Calculated
TOTAL PROTEINS	**6.50**	gm/dl	6.6-8.3	Biuret
ALBUMIN	3.50	gm/dl	3.5-5.3	BCG
GLOBULIN	3	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.17		1.0-2.0	Calculated
SERUM SODIUM	138.0	mmol/L	133-147	ISE
SERUM POTASSIUM	4.30	mmol/l	3.50-5.20	ISE
SERUM CALCIUM	9.0	mg/dl	8.6-10.3	Arsenazo III

Urea is the end product of protein catabolism and Urea undergoes filtrations at the glomerulus as well as secretion and re absorption at the tubular level. But the level may also be affected by diet as well as certain non kidney related disorders.

Blood Urea Nitrogen can be easily calculated from the serum urea level and the serum urea levels can be easily converted to BUN by multiplying it by 0.47.Creatinine is a small tripeptide found in the muscles and is simply used as a marker of renal function. Serum creatinine is a better indicator of renal function and more specifically glomerular function than urea. Creatinine is filtered at the glomerulus and its reabsorption at the tubular level is insignificant. Because of this creatinine clearance can be used to measure Glomerular Filtration Rate (GFR).

Uric acid levels are typically 3.5-7.2 milligrams per deciliter (mg/dL) in males and 2.6–6.0 mg/dL in females. Low levels are rare, but high levels can lead to gout, kidney disease, and other health issues.

Sodium is the main determinant of **plasma osmolality**, and serum levels are closely related to hydration (volume) status. Potassium is stored almost entirely **intracellularly** and is **excreted by the kidneys.**

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Visit ID	: **RCI24354**	Registration	: 23/Jan/2026 03:08PM
UHID/MR No	: RCI.0000021563	Collected	: 23/Jan/2026 03:09PM
Patient Name	: [redacted]	Received	: 23/Jan/2026 03:13PM
Age/Gender	: 58 Y 0 M 0 D /M	Reported	: 23/Jan/2026 05:01PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	:	Client Code	: 320
Client Add	:	Barcode No	: 10082595

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method

Derangements in potassium levels cause **myocardial instability**, leading to risks of potentially fatal arrhythmias.

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Visit ID	: **RCI24354**	Registration	: 23/Jan/2026 03:10PM
UHID/MR No	: RCI.0000021563	Collected	: 23/Jan/2026 03:12PM
Patient Name	: [redacted]	Received	: 23/Jan/2026 03:13PM
Age/Gender	: 58 Y 0 M 0 D /M	Reported	: 23/Jan/2026 05:01PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	:	Client Code	: 320
Client Add	:	Barcode No	: 10082595

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST				
Sample Type : SERUM				
TOTAL BILIRUBIN	0.82	mg/dl	0.1-1.2	Diazotized, Sulfanilic
CONJUGATED (D. Bilirubin)	0.22	mg/dl	<0.3	Jendrassik & Grolf
UNCONJUGATED (I.D. Bilirubin)	0.6			
SGOT	20	U/L	<35	Enzymatic,IFFC
S.G.P.T	23	U/L	0-45	Enzymatic,IFFC
ALKALINE PHOSPHATASE	91	U/L	30-120	Kinetic, IFCC
TOTAL PROTEINS	**6.50**	gm/dl	6.6-8.3	Biuret
ALBUMIN	3.50	gm/dl	3.5-5.3	BCG
GLOBULIN	3	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.17		1.0-2.0	Calculated
GGT	27	U/L	<55	Colorimetric Method

Note- 1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.

2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.

3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

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Visit ID	: **RC124354**	Registration	: 23/Jan/2026 03:10PM
UHID/MR No	: RC1.0000021563	Collected	: 23/Jan/2026 03:12PM
Patient Name	: [redacted]	Received	: 23/Jan/2026 03:13PM
Age/Gender	: 58 Y 0 M 0 D /M	Reported	: 23/Jan/2026 05:01PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	:	Client Code	: 320
Client Add	:	Barcode No	: 10082595

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
SERUM AMYLASE				
Sample Type : Serum				
SERUM AMYLASE	68	IU/L	0-80	KINETIC

Comments:

Amylase is produced in the Pancreas and most of the elevation in serum is due to increased rate of Amylase entry into the blood stream / decreased rate of clearance or both. Serum Amylase rises within 6 to 48 hours of onset of Acute pancreatitis in 80% of patients, but is not proportional to the severity of the disease. Activity usually returns to normal in 3-5 days in patients with milder edematous form of the disease. Values persisting longer than this period suggest continuing necrosis of pancreas or Pseudocyst formation. Approximately 20% of patients with Pancreatitis have normal or near normal activity. Hyperlipemic patients with Pancreatitis also show spuriously normal Amylase levels due to suppression of Amylase activity by triglyceride. Low Amylase levels are seen in Chronic Pancreatitis, Congestive Heart failure, 2nd & 3rd trimesters of pregnancy,
Gastrointestinal cancer & bone fractures.

SERUM LIPASE				
Sample Type : Serum				
SERUM LIPASE	82	U/L	13.0-60.0	Enzymatic

COMMENTS:

Lipase measurements are used in the diagnosis and treatment of diseases of the pancreas, such as acute pancreatitis, chronic pancreatitis, and obstruction of the pancreatic duct. In acute pancreatitis serum lipase activity tends to become elevated & remains for about 7 - 10 days .Increased lipase activity rarely lasts longer than 14 days, and prolonged increases suggest a poor prognosis or the presence of a cyst. Serum lipase may also be elevated in patients with chronic pancreatitis, obstruction of the pancreatic duct and non pancreatic conditions including renal diseases, various abdominal diseases such as acute cholecystitis, intestinal obstruction or infarction, duodenal ulcer, and liver disease, as well as alcoholism & diabetic keto-acidosis & in patients who have undergone endoscopic retrograde cholangiopancreatography. Elevation of serum lipase activity in patients with mumps strongly suggests significant pancreatic as well as salivary gland involvement by the disease.

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Visit ID	: **RCI24354**	Registration	: 23/Jan/2026 03:10PM
UHID/MR No	: RCI.0000021563	Collected	: 23/Jan/2026 03:12PM
Patient Name	: [redacted]	Received	: 23/Jan/2026 03:13PM
Age/Gender	: 58 Y 0 M 0 D /M	Reported	: 23/Jan/2026 05:01PM
Ref Doctor	: Dr.COMMANDER HOSPITAL	Status	: Final Report
Client Name	: COMMANDER HOSPITAL	Client Code	: 320
Client Add	: SHIV VIHAR WEST, HASTAAL	Barcode No	: 10082595

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
C-REACTIVE PROTEIN (CRP)				
Sample Type : SERUM				
C-REACTIVE PROTEIN	**38.60**	mg/L	0.0 - 5.0	TURBIDOMETRY

INTERPRETATION:
CRP is one of the proteins commonly referred to as acute phase reactants. CRP is distinguished by its rapid response to trauma or infection. CRP levels returns to normal quickly at the end of the acute episode. Measurement of CRP aids in evaluation of the amount of injury to body tissues.

Increased In:
Acute inflammation, Rheumatoid arthritis, lupus, Cardiovascular disease, atherosclerosis, Oral contraceptives, Inflammatory bowel disease, Giant cell arthritis, Osteomyelitis, Cancer of the lymph nodes, Pregnancy.

Decreased In:
Patients treated with carboxypenicillins, Liver failure

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